

TIMS GP Wrist and Hand Guide detailed management guidance

GP Management					
Condition / Diagnosis	Symptom(s)	Recommended management	Imaging guidance	When to refer to TIMS	When to refer to secondary care
Wrist Osteoarthritis	- Peak incidence aged 45 years or more - History of inflammatory arthropathy or trauma - General ache over wrist joint - Reduced ROM and function	- Analgesia - Intermittent use of splints – not continuous - Encourage exercise – see patient info - Rarely requires surgery Patient Information TIMS – Management of Hand and Wrist Pain Versus Arthritis Hand and Wrist Pain	X-Ray if severe pain and restricted function	- Diagnostic uncertainty - If patient has not responded to recommended management despite good compliance and understanding	Not indicated – referral via TIMS
Carpal Tunnel Syndrome	- Paraesthesia +/- numbness and pain in a median nerve distribution and radial side of the ring finger - Reduction of grip strength and reduced hand function - Typically worse at night & can awake from sleep - More common in women than men	- Ensure not related to DM or Thyroid Disease - Night splints 6 weeks (neutral position) +/- steroid injection (if able- or if not, refer to TIMS) Patient Information TIMS – Management of Carpal Tunnel Syndrome British Society for Surgery of the Hand Carpal Tunnel Syndrome Versus Arthritis Carpal Tunnel Syndrome	Not indicated	- Diagnostic uncertainty - If patient has not responded to recommended management despite good compliance and understanding	- Progressive neurological deterioration - Acute symptoms in relation to recent wrist fracture
De-Quervain's Syndrome	- Pain, maybe severe on radial side of wrist. - Pain aggravated by gripping, ulnar deviation of wrist. - Swelling in first extensor compartment - Crepitus in first extensor compartment on wrist/thumb movement - Commonly misdiagnosed as CMC arthritis	- Analgesia - Intermittent use of splint (Thumb Spica) – not continuous - Steroid injection (if able- or if not, refer to TIMS) Patient information TIMS Management of De Quervain's Syndrome British Society for Surgery of the Hand De Quervain's Syndrome Versus Arthritis Hand and Wrist Pain	Not indicated	- Diagnostic uncertainty - If patient has not responded to recommended management despite good compliance and understanding.	Ongoing symptoms despite recommended management

Finger Joint Arthritis	• Peak incidence aged 45 years or more • Pain and/or deformity and/or stiffness mainly in DIPJs /PIPJs. Not as common in MCPJs. • Presence of mucous cysts, Heberden's nodes and/or Bouchard's nodes.	• Analgesia • Encourage exercise – see patient info Patient Information • TIMS – Management of Hand and Wrist Pain • British Society for Surgery of the Hand Terminal Joint Arthritis	X-Ray if severe pain and restricted function	• Diagnostic uncertainty • If patient has not responded to recommended management despite good compliance and understanding.	Not indicated – referral via TIMS
Dupuytren's Disease	• Peak incidence in men aged 50 years or more; or women aged 60 years or more – more common in men • Skin thickening, pitting or nodules in palm. • Fibrous cords in in palm and/or digits • Flexion deformity at MCPs and/or PIPs	• Analgesia • Advice to use hand as normally as possible. • Reassure that tender nodules likely to become pain-free with time. Patient Information • British Society for Surgery of the Hand Dupuytren's Disease • NHS Dupuytren's Disease • Versus Arthritis Hand and Wrist Pain	Not indicated	• Diagnostic uncertainty	• If patient has contracture <30 degrees at MCP or 20 degrees at PIPJ and symptoms affecting ADL's.
Trigger Digits	• Possible history of pain • Stiffness and clicking when the finger/thumb is moved – especially first thing in the morning • Tender A1 pulley; but fully mobile digit • Difficulty extending digit: digit may get stuck in a bent position then suddenly pop straight • Loss of complete active flexion and/or fixed contracture if digit locked	• Encourage maintenance of normal movement with care when doing activities that cause clicking. • Spontaneous recovery may occur with time. • Steroid injection (if able- or if not, refer to TIMS) Patient Information • TIMS Management of Trigger Digit • British Society for Surgery of the Hand Trigger Digit • Versus Arthritis Hand and Wrist Pain	Not indicated	• Diagnostic uncertainty • If patient has not responded to recommended management despite good compliance and understanding.	• If concerned that digit will become permanently locked • If digit is permanently locked • Ongoing symptoms despite recommended management

Ganglion	- Usually located on dorsal or volar-radial aspect of the wrist or volar aspect base of finger (e.g. pearl ganglia) - Digital mucous cyst occurs over the dorsal SIDE of the DIPJ (ass. with OA) - Ganglia will transilluminate, fluctuate in size, smooth, and round in appearance.	- Advice on nature of condition. - If asymptomatic Ganglia: no treatment required. - Education of avoidance of aggravating postures/activities Patient Information British Society for Surgery of the Hand Ganglion Cyst	Not indicated	Diagnostic uncertainty	If significantly painful and affecting ADL's consider referral to secondary care.
Thumb Base Arthritis	- Peak incidence aged 45 years or more - Functional difficulty in gripping - Pain at thumb CMC joint/radial side of wrist - Crepitus on movement of thumb - Deformity of thumb (not always present); adduction deformity first ray"squaring" of CMC joint	- Analgesia - Reassurance and encouragement of normal function - Encourage exercise – see patient info - Optimal analgesia - Activity modification/pacing/ joint protection - splint use (not continuous). - Steroid injection steroid injection (if able- refer TIMS if not) Patient Information TIMS Management of Thumb Pain TIMS Management of Carpometacarpal Thumb Joint OA British Society for Surgery of the Hand Basal Thumb Arthritis	X-Ray if severe pain and restricted function	- Diagnostic uncertainty - If patient has not responded to recommended management despite good compliance and understanding	Not indicated – referral via TIMS